Emergency Department Note

SYNTHETIC DOCUMENT - generated for software testing, not a real patient record

Case ID	SYN-0009	**Age / Sex**	22 / M
Setting	Emergency Dept	**Primary dx**	Bacterial meningitis

Chief complaint

Fever, headache, and confusion for 24 hours, now difficult to rouse.

History of present illness

University student in shared housing. Flu-like illness for 2 days, then severe headache, photophobia, vomiting, and a rash on the legs noticed by roommate this morning. Roommate reports confusion since this afternoon.

Past history / medications

No medical history, no immunizations on record beyond childhood schedule. No recent travel.

Examination

Temp 39.6 C, HR 128, BP 92/50, CRT 4 s. Non-blanching purpuric rash on legs and trunk. Marked neck stiffness, positive Kernig sign. Pupils equal, reactive. No focal deficit.

Glasgow Coma Scale

Eyes open only to pain. Confused, speaks few coherent words. Motor: withdraws from pain, no consistent command following.

GCS on arrival	Score	Response
Eye opening (E)	2	To pain
Verbal (V)	4	4 - Confused
Motor (M)	4	Withdraws from pain
TOTAL GCS		**10**

Investigations

Blood cultures x2 drawn. Lactate 4.1, CRP 280, WBC 22. Lumbar puncture deferred until after CT given depressed GCS: CT head no mass effect. CSF: turbid, WBC 3200 (95% PMN), protein 4.2 g/L, glucose 0.6 mmol/L; Gram stain with Gram-negative diplococci.

Assessment

Meningococcal meningitis with sepsis (GCS 10, purpura). Public health notification required.

Plan / disposition

Ceftriaxone 2 g IV within 15 min of arrival, dexamethasone 10 mg IV, fluid resuscitation 30 mL/kg, ICU admission. Droplet isolation, prophylaxis for close contacts, notification to public health authority.